

NW London Outpatient Pathways

Abnormal LFT (1/2)

Note: The degree of abnormality is not an indication of the severity of disease

V1.6 Oct2019

Consider stopping any causative drugs
Contact local liver team if advice required

History

Alcohol history (>14 units/week) / Metabolic syndrome and BMI
Drug history / Risk factors for viral hepatitis

Suspected alcohol cause:
Follow alcohol guidelines

Hepatic liver enzymes
↑ ALT or AST

Synthetic failure

Jaundice, low albumin, prolonged
INR

or

Suspected Malignancy

Weight loss
Marked Cholestasis

Isolated raised bilirubin with
otherwise normal liver blood results

Liver aetiology screen

AST, GGT, FBC, INR
Ultrasound
Hep B surface antigen
Hep C antibody
Autoantibodies (ANA, AMA, ASMA, LKM)
Immunoglobulins (IgA, IgG, IgM)
Ferritin
Transferrin saturation (for elevated ferritin only)
HbA1C (if no recent results)
Lipids (if no recent results)

Consider:

- 2WW referral to hepatology
- Contact jaundice hotline
- If considering emergency admission contact accepting team or AMU/AAU

Most commonly due to Gilbert's
syndrome (unconjugated
hyperbilirubinaemia)

Less commonly due to haemolysis
(consider reticulocyte count, LDH,
haptoglobin)

Repeat LFTs with split bilirubin
and FBC

Consider reticulocyte count and
LDH for haemolysis

USS is either normal or has fatty
liver changes

With risk factors

T2DM
BMI>25
Dyslipidaemia
Hypertension

Follow NAFLD pathway

Note: Patients with NAFLD may
still be prescribed statins

Normal USS
Normal liver aetiology screen
No NAFLD risk factors

ALT and/or AST remain
abnormal

Abnormal USS (not NAFLD)
+/- positive liver aetiology
screen

Refer to hepatology

Gilbert's Syndrome

Diagnose Gilbert's if

- Total bilirubin <85
- Conjugated fraction <20% total
- Normal FBC / no haemolysis
- Other LFTs normal

Confirm diagnosis to patient
Provide information

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Abnormal LFT (2/2)

Note: The degree of abnormality is not an indication of the severity of disease

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